

OSCAR HEALTH INC (OSCR)

August 27, 2026 - 5:34pm EST
by

tallinterest

2026 2027

Price:	31.23	EPS	0	0
Shares Out. (in M):	308	P/E	0	0
Market Cap (in \$M):	9,640	P/FCF	0	0
Net Debt (in \$M):	-3,644	EBIT	0	0
TEV (in \$M):	6,342	TEV/EBIT	0	0
Borrow Cost:	General Collateral			

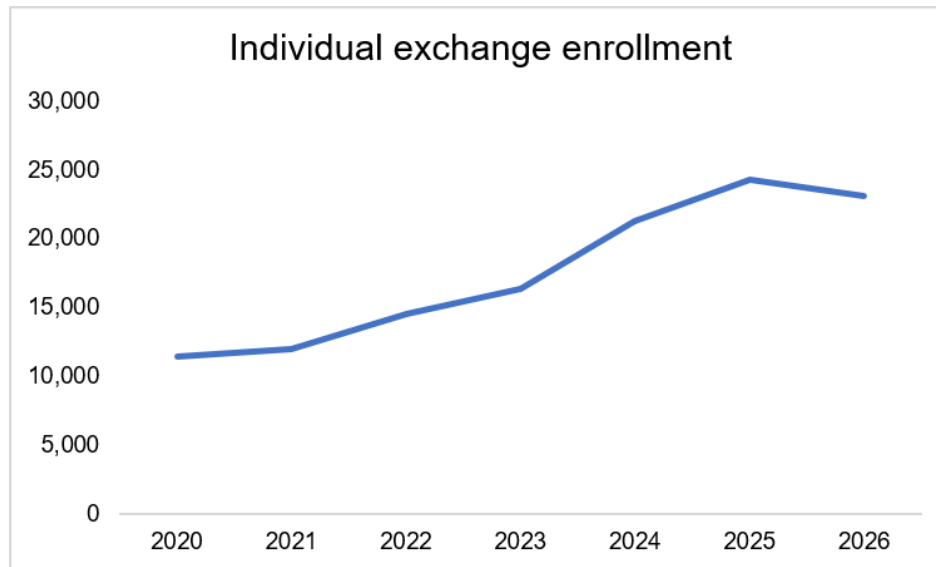
Description

Business overview: Oscar Health (NYSE:OSCR) is a ~\$10B market cap insurer currently trading at ~18x 2026 EPS. Over the next 6-18 months, I see one of or a combination of the following: (1) missing profitability guidance stemming from a structurally sicker patient population and ACA risk adjustment mechanisms; (2) Losing patients at a faster rate than currently anticipated, leaving the business with less premiums and a riskier book. Street is at \$1.68 in EPS, implying ~82% MLR and valuing the business at around 18x 2026 EPS. I believe this business will end up trading at somewhere closer to 15x P/E on a base of ~\$0.75 EPS at the end of this year, implying a 65% downside to current price.

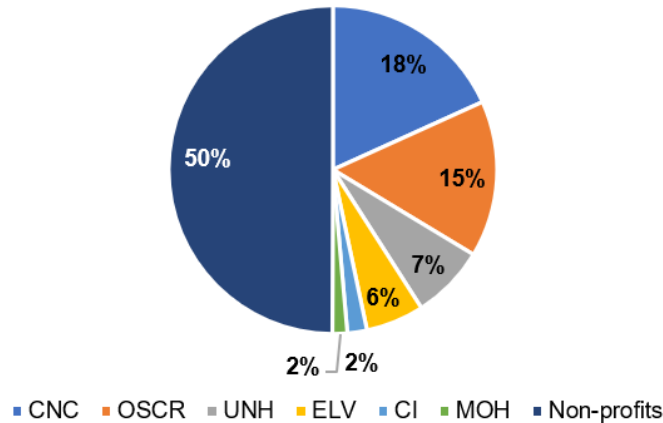
OSCR is a pure-play insurer under the ACA exchange market with ~100% of enrollees covered under individual / small groups. The company was founded in 2012 and currently serves ~3mm enrollees, primarily in FL, TX, and GA (~60% of book).

Mark Bertolini is CEO of OSCR and took over from co-founder Mario Schlosser (now president of technology). Mark was formerly the CEO of Aetna from 2010 through the CVS sale in 2018, with the company's stock CAGR'ing at ~30% p.a. during that period as the industry saw massive tailwinds from Medicare. Although Aetna as a whole saw huge success, their individual exchange market business was quite a flop. Aetna lost ~\$700mm on the exchange in a few years, pulled out of most states in 2016 and 2017, and then completely exited in 2018 right before the business sold to CVS. In Feb. 2017 at the WSJ Future of Healthcare event, Bertolini even claimed that the individual exchange market was "in a death spiral," and that the law did not contain enough money to cover the population it served. After spending a few years as co-CEO of Bridgewater, Mark joined OSCR in 2023, following the introduction of heavy subsidization that drastically improved the market's attractiveness.

Industry overview: The individual exchange market was established in 2014 under the ACA aka Obama Care. This segment of the ACA was formed to provide insurance to people without access to traditional employer plans (self-employed, small business owners, gig workers, etc.). A few for-profits hold roughly half of share (CNC, OSCR, UNH, ELV, CI, MOH) while the rest of demand is served by non-profit private payors (The Blues, regional systems, and Medicaid-focused nonprofits). Notably, OSCR is the only pure-player in this space. Every other provider has multiple insulating business lines in other segments which can absorb losses from their individual exchange books. CNC has the most meaningful book of non-pure-players, with the individual book accounting for ~13% of revenue.



2026 carrier market share



There are a few key mechanisms that govern the individual exchanges under ACA:

[1] Guaranteed issue: All carriers must take all comers who sign up for their plans. Carriers can only price annually and cannot refuse coverage for patients who are too sick.

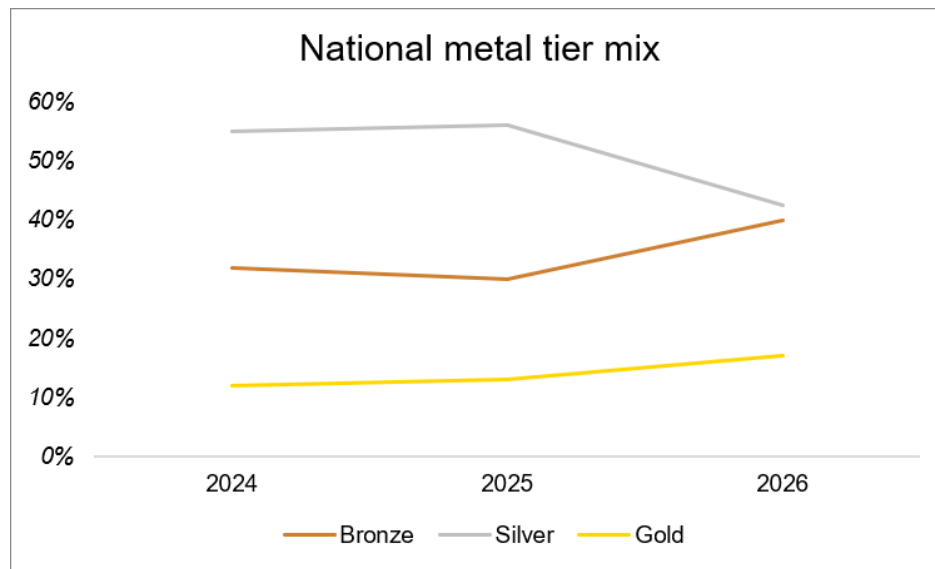
[2] Tax credits: Through the Center for Medicare & Medicaid Services (CMS), individuals signed up for individual exchange plans are eligible to receive Advance Premium Tax Credits (APTCs), which are direct tax subsidies for premiums. APTCs are calculated based on the benchmark premium for the second-lowest cost silver plan in a region minus a MAGI-based contribution percentage. Typically, payors would also load their cost-sharing reduction (CSR) costs into silver plans for APTC benchmarking, leading to relatively lower-cost bronze and gold tiers which became cheap or free after subsidization. APTCs only apply to individual insurance members with MAGI between 1-4x the poverty line.

[3] Risk adjustment: To further incentivize insurers to take on sick patients, CMS offers a zero-sum transfer program within each state. The transfer mechanism assigns risk scores to payors based on their coded diagnoses (codes submitted by HCPs), and payors with risk lower than the state average score pay into the insurance pool while those with above state average scores get a cash receivable. Transfers for any single benefit year are not settled until June of the following year, causing payors to have to give a periodic estimate as to what they think they will pay or receive from the pool.

[4] Claims: The ACA requires all individual exchange insurers must pay out at least 80% of their net premiums as claims on a 3-yr. rolling average. If an insurer falls short, they must rebate their members the following fall.

Additionally helpful for background on all healthcare insurance is a brief breakdown of tiers. Insurers offer different tiered plans available to customers, each with their own scaling premium and deductible. The premium is the monthly fee patients pay which can be thought of as a subscription charge, while the deductible is the dollar amount a patient must pay before they can start receiving insurance coverage on their treatments. Tiers are categorized by metals (gold, silver, bronze) with gold plans having the highest premium and lowest deductibles and bronze having the lowest premium and highest deductibles.

For example, let's say Joe has a bronze plan with \$100 monthly premium and \$5,000 deductible. He knows he will need knee replacement surgery at some point within the year costing \$10,000. Each month, Joe will fully pay the \$100 premium and whatever medical costs he has until he has spent a total of \$5,000 out-of-pocket. After that, Joe might decide to get his knee surgery which he can now submit for an insurance claim to receive coverage for.



The final important note for this industry is that it's not a compounder, unlike what mgmt. / longs will advertise this business as. The growth algo is premiums x members * (1 - MLR - SG&A%) ~ EBIT. Although some SG&A leverage can be realized, it is the only term that compounds; gross margin is capped by the 80% rebate floor, so there is no clear advantage to growing patients is the chance for higher \$ profit. Margins do not improve structurally with growth, and since the patient population is capped by subsidy appropriations, there also isn't a clear path to gaining share outside of undercutting net price against the benchmark silver, inheriting exits, or paying the broker channel, all margin-dilutive. Additionally, price cuts attract healthier price-shoppers who inherently increase the risk payable for the payor.

Setup:

[1] Subsidy expiration: Through regulatory change within the ACA's individual exchange market and resulting mechanical shifts in metal tier mix, the market has become structurally sicker as younger, healthier, and more elastic customers exit.

- A. After COVID, ARPA and the Inflation Reduction Act led to the introduction of eAPTCs (enhanced APTCs), an additional subsidy on top of previous APTCs to directly fund the premium payments patients would pay out for individual insurance. This eliminated or severely reduced premiums for many, removing the 4x poverty line cliff and capping premium payments at 8.5% of any income level. The additional credits were eliminated at the end of 2025, alongside legislation which beefed up eligibility and verification regulation (documented income verification, restricted special-enrollment periods, \$5 auto-enrollment charge, etc.).
- B. Alongside customer exits, the industry has seen an exodus into high-deductible bronze tier plans (+10pts) with significantly lower claims visibility for insurance providers. Patient spending burns through deductibles by 2H, so insurer-paid claims are loaded into the rear portion of a billing year. Members who don't end up meeting their deductible but are still sick also don't show up in risk codes as they have fewer encounters, making the patient base show up as healthier than they might actually be.

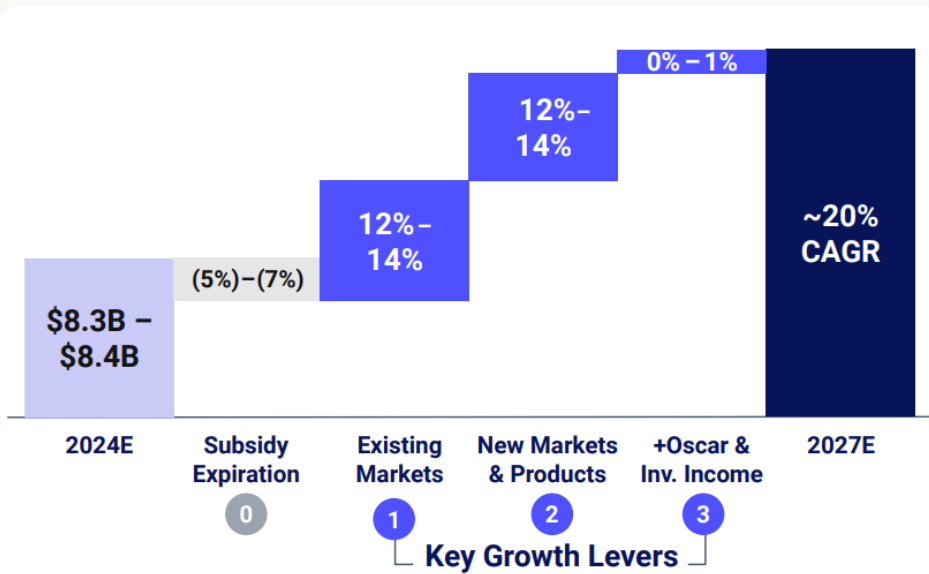
[2] Risk mechanics: Constrained by market regulation, payors in the individual market are left operating within a maximum 5% operating margin industry with low visibility and high volatility of PnL.

- C. The ACA requires all individual exchange insurers must pay out at least 80% of their net premiums as claims on a 3-yr. rolling average. If an insurer falls short, they must rebate their members the following fall. When you account for SG&A expense, payors are mechanically capped at ~5% operating profit margins.
- D. The ACA also requires a risk adjustment mechanism to incentivize insurers to take in all patients regardless of health. Payors with sicker enrollees get an annual receivable and healthier payors fund this receivable. The payment is measured in each state and is effectively a spread between market avg. health and payor health multiplied by state avg. premium (see appendix 1). Each quarter insurers estimate what their risk is based on their internal data and 3P data vendors, with the estimation acting as a contra revenue. This item is subject to high volatility and is extremely difficult to estimate. Furthermore, insurers only get their actual \$ figure in Q2 of the following year, creating timing challenges.

[3] Industry exits: As a result of points [1] and [2], multiple players have either shrunk their books or completely left the industry. The largest payors (Aetna, UNH, CNC) have either completely exited or shrunk their books through price hikes. These customer books have since either exited the market entirely or left to other insurers. OSCR alone added ~1mm at the start of 2026 (taking up ~30% of total exit volume), with mgmt. claiming strength in underwriting relative to competition, although they took on so much volume due to relatively lower rates. Although seemingly a beneficiary of new customer additions, OSCR has no previous data on these patients, making underwriting more difficult on a base that was already showing profitability struggles from their previous payors.

Growth Engine:

Multiple levers to drive sustained top-line growth



Key Strategic Levers to Drive ~20% Top-Line CAGR

- 0 Subsidy Loss & Enrollment Shift**
Market shrinkage and metal shift from enhanced subsidy expiration
- 1 Expand Existing Market Presence**
Balance rates with growth to mature in existing markets
- 2 Market & Product Evolution**
Enter new markets/states with innovative plan designs & capitalize on ICHRA opportunity
- 3 +Oscar & Investment Income**
Grow Campaign Builder and leverage cash position for investment income

OSCR published this chart in their 2024 investor day presentation decomposing their growth target of ~20% CAGR through 2027. Through the most recent quarter, the numbers have lived up to expectation, but for drastically different reasons than what's presented here.

[1] Existing markets growth is attributable to the one-time shift from recent exits.

New cohort		2025 cohort		2025 cohort retention	
1Q26 MM	9,523	1Q25 MM	6,118	88%	89%
2Q26 MM	9,206	2Q25 MM	6,100	90%	91%
1H26 MM	18,730	1H25 MM	12,218	92%	93%
(/) 1H26 direct premiums	\$11,697	(/) 1H25 direct premiums	\$6,870	103%	102%
1H26 PMPM	\$625	1H PMPM	\$562	102%	101%
New member PMPM skew	-5%	Renewal MM	10,996	101%	100%
New member PMPM	\$593	Renewal rate	90%	99%	98%
1H26 new MM	7,733			98%	97%
New member MM share	41%			97%	96%
1H26 new member direct premium	\$4,587.96			96%	95%
% of total	39%			95%	94%
				94%	93%
				93%	92%
				92%	91%
				91%	90%
				90%	89%
				89%	88%
				88%	87%

2026 contribution bridge	
Lapse of 2025 base	(\$687)
Renewal repricing \$ impact	\$925
Renewal PMPM	\$646
Renewal cohort 1H26 contribution	\$7,109
(+) new member direct	\$4,587.96
1H 26 direct	\$11,696.74
\$ Chg. In direct premiums y/y	\$4,826.25
Renewal contribution to growth	\$238.29
% growth from 2025 cohort	5%
% growth from new members	95%

In 1H25, OSCR had 12,218 existing member months (MMs ~ avg. members * months in period). I estimate ~41% of member months in 1H26 are attributable to new members, based on flexing renewal rate between 100% and 80% and taking the midpoint at 90%. I also assumed new members skewed ~5% lower PMPMs compared to the average, accounting for higher bronze mixes. Netting the impact of the 2025 lapse and renewal repricing, the existing 2025 cohort only contributed ~\$240mm of direct premium revenue growth out of \$4.8B in direct premium revenue growth, while the remaining 95% is attributable to members inherited from competitor exits.

Growing new customers is not repeatable in this fashion, and the mass exodus of competitors in the space has driven nearly all of OSCR's growth so far this year. These customers are also not as sticky as the renewal base, the majority being placed into OSCR via brokers vs. direct selection.

[2] New markets & products have also not seen huge success, and certainly not the 12-14% compounding growth engine mgmt. is citing here.

OSCR entered 0 states in 2025, and they've announced plans to enter / entered 2 in 2026, AL and MS. AL has dominant Blue Cross market share (90%+), and alongside MS OSCR has only entered in a cumulative 16/573 counties in both states. In terms of potential expansion into other states, OSCR's runway is fairly short. The material absences in the company's state portfolio are CA, SC, UT, LA, and IN, a total of 5 states.

OSCR exited CA in 2024, consistently recording MLR > 100%. SC, UT, LA, and IN are all Blues or integrated system dominated. These markets also have drastically low effectuation (a person is considered effectuated if they sign up and make their first payment), so they are dominated by competitors and generally unattractive. Market entrance is attractive when there is high-churn, subsidy-heavy / low income, and brokers drive transactions. OSCR already holds share in those markets, and would be left fighting for share in Blues states where network depth and provider relationships are far ahead for the incumbent.

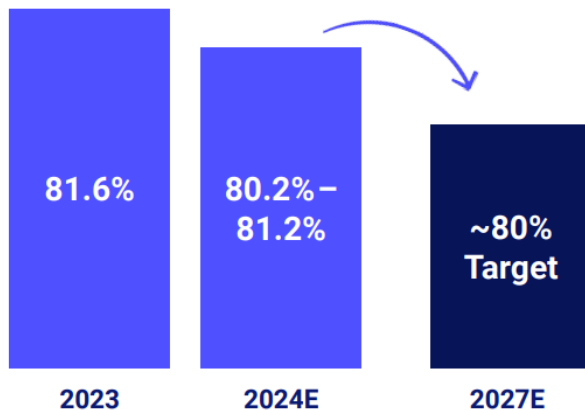
So if new states aren't an option, where does the growth come from? Mgmt. would tell you new growth will come from ICHRA (Individual Coverage Health Reimbursement Arrangement). ICHRAs work by giving employers of any size the option to reimburse their employees' tax-free for individual health insurance policies to buy on their own. ICHRA penetration is at a meager ~0.3% of employer coverage, for a few reasons: (i) ICHRA drops APTCs: ICHRA works best for employers with low-wage workforces, but for those exact workers their plans would end up being cheaper / more effective if they simply enrolled by themselves in the individual market and received APTCs; (ii) Individual exchange plans are worse: Employers and their employees would rather not switch to plans with fewer in-network providers, and individual premiums are age-weighted 3:1 vs group plans which are based on a firm-wide rating. Older employees are significantly worse off, and employers also have to assist their employees in navigating multiple individual plans across different metal tiers, creating administrative overhead; (iii) as 2026 has seen massive rate increases, ICHRA is no longer even viable from a monetary perspective. Although OSCR cites ~115mm patients as an addressable ICHRA market, there is no clear route to achieve this number, or get anywhere close for that matter.

OSCR's growth is inherently inorganic. They have no clear path to sustaining their growth outside of relying on competitor exits.

Margins:

We are driving a sustainable MLR as we continue to grow

Medical Loss Ratio



Disciplined Pricing Approach

Strategically balance rate actions with growth and retention targets as we mature markets and expand

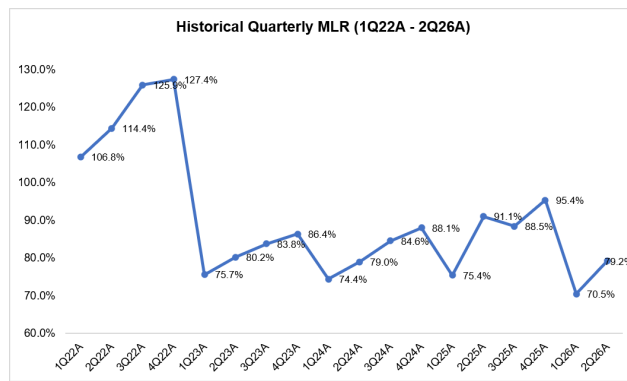
Medical Cost Savings

Execute technology-enabled affordability initiatives spanning core pillars of Total Cost of Care, including payment integrity efforts, site of care optimizations, and high-performance network routing

Scale

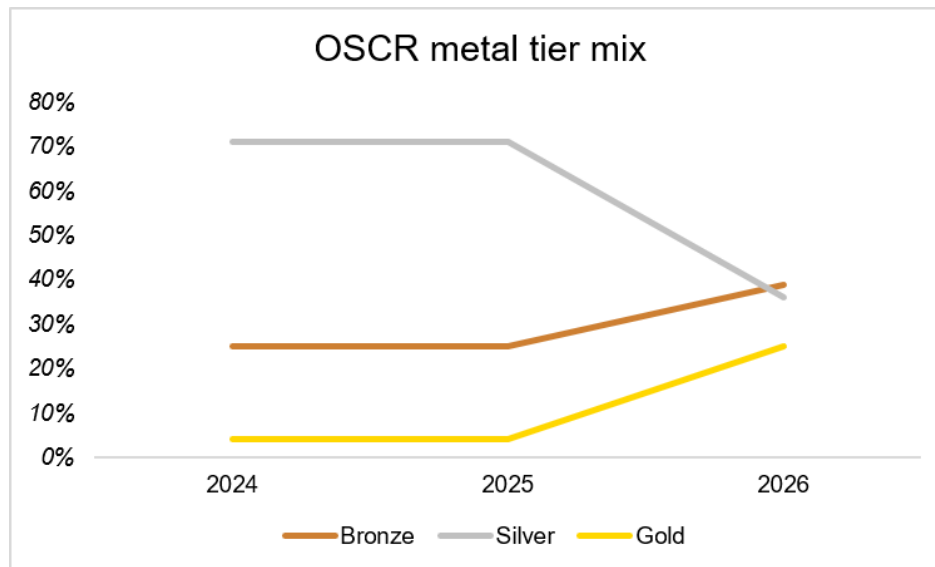
Reduce medical costs through strategic partnerships, including PBM agreements & key provider arrangements

OSCR also included this ~80% MLR guide for 2027 in their last investor day. I believe that there is no way to systematically keep MLR at ~80%, or even reduce it to 80% by 2027. OSCR MLR was a whopping 87.4% in 2025. This year, it's been tracking better, but still not very consistent. Assuming an MLR of 80% that can be systematically achieved implies that the payor can consistently maximize strong performance without going into rebate territory. As for all the levers mentioned on the slide, while they may be able to improve average MLR, they can not solve for the high variance consistently seen in this industry. In the 2 years since this investor day deck, OSCR has made no progress at significantly improving MLR.

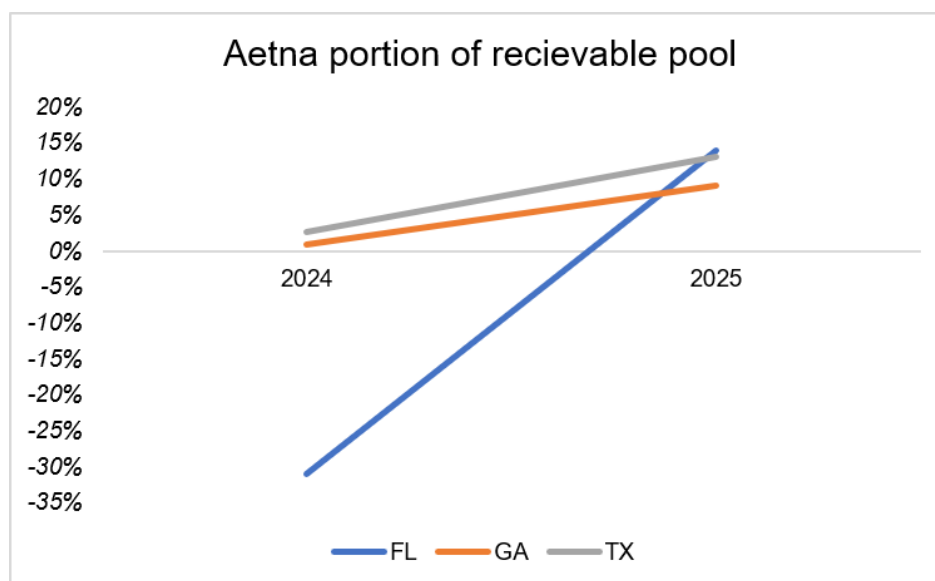


Thesis:

[1] OSCR guidance and consensus is pricing MLR at 81.75%. I believe there are multiple pathways to this being 125bps or higher, which would lead to a decline of ~60% in EPS. Street implied EPS is at \$1.68, which assumes the aforementioned ~82% MLR. Quarterly RA typically swings on changes in 3P data or significantly different-than-expected claims. Both claims and RA are being understated as the industry has shifted to lower visibility high-deductible enrollees. High deductible plans lead to patients pushing elective procedures and high ticket items into 2H, and while insurers have cited pricing for higher market morbidity, there has been no discourse of lower visibility as a result of bronze mix shift (increased to 39% from 25% vs. industry wide shift to 40% from 30%). Bronze plans were typically made up of healthier patients who preferred lower monthly premiums, but the metal has now been distorted by sicker patients attempting to insure themselves in any way possible. This leads to actuarial uncertainty, increasing the difficulty of modeling risk / claims for this plan tier. OSCR is adversely affected by this mix change when compared to competition, as it now makes up ~40% of members vs. 25% previously. The same actuarial issue occurs at the gold tier, with the highest premiums and claims. Here, OSCR previously held only 4% of patients, but that number has since spiked to ~25% (while the market mix is only around 17%). This high gold swing is significantly worse for OSCR claims, and further points as evidence that they are picking up sicker patients. This leaves OSCR with their highest patient base consisting of their least accurately priced tiers. Furthermore, the market exits leave OSCR with a base made up of ~45% new patients which are generally harder to model for risk and claims. A change in MLR of even 1% slashes EBIT by nearly a third from guided figures. Risk transfer is also sized off of statewide average premiums, which are up ~20% y/y, while OSCR's own premiums have compressed as members have moved to lower tiers, implying that risk adjustment errors are further mechanically unfavorable for OSCR.



A likely suspect for significant share donation to OSCR was formerly-Bertolini-ran Aetna. Aetna subscribers were auto-assigned to OSCR plans, and had significant presence in OSCR's main markets (FL, GA, TX). In 2024, Aetna added significant members in FL, GA, and NC, among other states as they were expanding rapidly from their 2022 re-entrance. Their fast-growing and younger book showed up as coding healthier, leading them to a net payable position into Florida in 2024. By 2025, however, Aetna went into a receivable position as healthy patients left in advance of their exits, leaving them with a sick population. Across all the other main markets, Etna grew into a deeper receivable position, showing the same effect with less drastic impact compared to FL. At the same time, 2025 for Aetna happened under the backdrop of shrinking their population base, leading me to believe they had some of the sickest members in the market (a primary reason to exit this market after eAPTC expiration). In FL in particular, Aetna actually flipped from a net payable position to being a receiver, implying significant deterioration in their patient population y/y, a book which OSCR has since inherited through hidden bronze plans as well as higher gold contribution.



Note: Aetna 2024 FL position was ~31% of payable pool, shown here as -31%

The other prime suspect of share donation, and likely larger, is CNC. CNC's own commentary points us in the right direction: mgmt. expecting its pricing actions to support "meaningful pre-tax margin expansion" (4Q25 call); CEO Sarah London said the business would see "a reduction in the footprint where we are a low-cost bronze player" (4Q25 call). If CNC is repricing for margin expansion, and getting rid of their bronze footprint, that must lead us to believe that their bronze segment was not performing very well. Again, this is a book that OSCR has now picked up for themselves.

So if OSCR's MLR goes up, shouldn't it be covered by the RA? Although the mechanism was established to proportionally offset the impacts of taking on a sicker population, in reality the move is almost never 1:1. The reason is because RA awards from CMS are based on coded figures, specific diagnoses codes that are submitted by healthcare providers for their patients. Claims are often undercoded, however, and so functionally many claims arrive without codes, increasing MLR without shrinking the payable gap, leading to two sources of loss on the same premium \$. Industry experts cite carriers on average missing 20%+ of chronic conditions when submitting claims.

[2] Channel checks have pointed to customer exits running greater than initially estimated, which will lead to faster than expected customer exits and revenue coming in materially lower than consensus. Street is currently implying \$18.8B revenue which implies only 2-4% customer churn through the end of the year. OSCR mgmt. Already raised their initial decline projection of 1-2% to 2-4% as they saw more slippage after expiring payment grace periods. An expert tracking ACA since the inception of the program noted expected enrollment declines closer to the tune of ~6mm exits vs. the ~2mm currently expected. Not only would this significantly pressure topline (~7% revenue drop from guidance / consensus), but it also leaves OSCR with sicker patients and a higher MLR. 2027 consensus figures also model OSCR growing revenue to ~\$20B, which doesn't happen without either (i) rate increases that would confirm 2026 being underpriced for risk; or (ii) membership growth, which comes in this market through exits which naturally function under adverse selection. As previously mentioned, the 2024 investor day highlighted end market growth as a key driver for this business moving forward. With signs pointing that regulation is leading the market to shrink, and maybe even revert to its pre-eAPTC enrollment position, there is no runway for OSCR to grow in their existing markets outside of betting on more exits to come.

Churn is also likely concentrated in the newer cohort, particularly in FL. The ASPE (Assistant Secretary for Planning and Evaluation of the HHS) has estimated improper / phantom / fraudulent enrollment peaking at ~5.6mm in 2025, driven by brokers placing enrollees into \$0-premium plans. OSCR sells both bronze and gold at \$0 net premium to subsidized enrollees, as well as sources 90-95% of members through brokers. CMS's verification rules regarding individual exchanges are targeting exactly this. For OSCR, crackdowns remove high-margin members who never submit any claims but pay premium \$. A 5% purge of such members lifts MLR roughly 425bps with no change in underlying health.

Valuation:

[1] My reward case contemplates both thesis points playing out, leading to \$0.75 EPS, implying revenue coming in at \$17.6B (7% variance from street) and MLR of 83% (125bps higher than street). I then applied a 15x P/E multiple yielding \$12 share price and 65% downside to current.

[2] My risk case assumes management achieving their revenue target and hitting peak operating margins of ~5% through a lower-than-expected MLR of 80%. With these figures, I got to \$2.55 in EPS and applied 18x P/E, yielding a \$45 share price and 38% upside to current. This represents a 1.7x R/R.

Recommendation: I recommend placing short position on OSCR, monitoring for **[1]** Rate amendment filings to be finalized in early September, ideally showing 2027 rate increases stemming from higher-than-expected morbidity rates / claims; **[2]** Q3 payor and hospital earnings showcasing high claims, high morbidity, and high utilization; **[3]** Enrollment data tracking towards ~6mm exits vs. ~2mm; **[4]** Final CMS report release in 2Q27 earnings, likely with a forced risk accrual revision placing OSCR in a deeper payable position. OSCR has an upcoming investor conference in September, I think it's plausible to see them start talking down the numbers then to soften the blow into a harsher Q3 print.

Risks:

[1] Regulatory: Strong democrat positions after the midterm in congress could lead to reinstatement of eAPTCs and subsequent multiple expansion for OSCR. Even if regulation is headed this way, however, it would either happen in '27 or post-Presidential elections in '28.

[2] Market health: If the overall market / CMS reads for coded risk show up generally healthier this decreases OSCR's net payable position on the risk adjustment and would boost operating profits. Data in 1H showed up slightly favorable but OSCR mgmt. didn't revise, giving them some insulation. Based on channel checks I don't believe the data will continue to show favorably, and experts cite Wakely reads as becoming less dependable each year.

[3] OSCR patient quality: If the majority of OSCR's new patient intake do not present high morbidity, OSCR could get away with achieving guided MLR. I believe my thesis points show that this is unlikely, but the data has not ruled it out yet.

Appendix:

1. Risk adjustment calculation

2. RA error impact

		Misscoded rate					
		0% 1% 2% 3% 4% 5%					
OSCR risk score (vs. mkt benchmark of 1.0)	0.8						
Gap vs. mkt	0.2	>> based on 20% premiums as RA					
		40%	\$0	(\$75)	(\$150)	(\$225)	(\$301)
		41%	\$0	(\$77)	(\$154)	(\$231)	(\$308)
Leverage on OSCR error	4.0x	>> = OSCR risk score / gap					
Leverage on mkt / 3P error	5.0x	>> = Mkt risk score / gap					
		42%	\$0	(\$79)	(\$158)	(\$237)	(\$316)
		43%	\$0	(\$81)	(\$162)	(\$242)	(\$323)
		44%	\$0	(\$83)	(\$165)	(\$248)	(\$331)
		45%	\$0	(\$85)	(\$169)	(\$254)	(\$338)
New cohort share of member months	41%	New cohort MM share					
(x) Misscoded rate	2%						
Book-level OSCR error	0.83%						
Transfer error	3.3%	>> = Book-level OSCR error * OSCR leverage					
(x) 2026E RA	(\$4,696)						
True-up	(\$155)						

3. Proposed rate change log

Section 2: Proposed Rate Change(s)

BCBSM is filing a year-over-year average rate increase for 2027 for all individual products that were offered in 2026 of 12.93%. Significant drivers of the rate change include:

Factor	Rate Impact
2026 Trend and Morbidity	-4.4%
2027 Trend and Morbidity	9.8%
Risk Adjustment	1.1%
Geographic coverage impact*	-2.1%
Retention Impact	1.7%
Catastrophic Factor Impact	1.3%
Other	0.1%
May 27th Filing	7.19%
Updated 2025 Experience	-0.2%
Updated Morbidity and Risk Adjustment	0.4%
Updated 2026 Trend	5.6%
Insurance Provider Assessment	0.1%
Other	-0.6%
July 13th Filing	12.93%

BCBS of Michigan including amendment language around "Updated 2026 Trend" (Jul-26)

4. Plan enrollment splits

Oscar Health				
Membership information		Actual	Actual	Actual
Period		FY24A	FY25A	1Q26A 2Q26A
Avg. members		1510	2011	2608 3069
Bronze				
Enrollment split		25%	25%	39% 39%
Member count		377	503	1017 1197
Silver				
Enrollment split		71%	71%	36% 36%
Member count		1072	1428	939 1105
Gold				
Enrollment split		4%	4%	25% 25%
Member count		60	80	652 767

OSCR plan enrollment splits (FY24 - YTD26)

5. Consensus estimates

Oscar Health			
Street estimates		Forecast	Forecast
Period		FY26E	FY27E
Revenue		\$18,742	\$20,294
Y/Y Growth %		60%	8%
		0.34x	0.31x
EBIT		\$659	\$772
Margin %		3.5%	3.8%
Implied Multiple		9.62x	8.22x
EBITDA		\$744	\$846
Margin %		4.0%	4.2%
Implied Multiple		8.52x	7.49x
EPS		\$1.68	\$1.88

OSCR Consensus estimates (FY26 - FY28)

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Catalyst

Investor day
Earnings
3P Data release

Messages

No messages